INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Gemcitabine alone: nasopharyngeal cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00514
Version 3

> Days 1 and 8; every 21 days; up to 6 cycles.

Treatment stages

01 **Chemotherapy course**
Days 1 and 8; every 21 days; up to 6 cycles.

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	4
03	Symptoms and supportive care	8
04	Sources and further information	15

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Head and neck: nasopharyngeal cancer

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Fever, shivering, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink needs urgent oncology advice. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112/999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

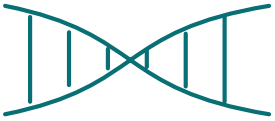
What each medicine does

Gemcitabine is used to control the cancer and relieve symptoms. Your team discusses expected benefit, alternatives and when to review whether treatment is helping.

ANTIMETABOLITE CHEMOTHERAPY

Gemcitabine

intravenous (IV) infusion



Interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) synthesis

Gemcitabine is converted inside cells into forms that disrupt DNA building blocks and DNA copying. It can affect normal blood-cell production as well as cancer cells.

Activated inside cells → DNA production disrupted → Cell division slowed

What to watch for

Report fever, unusual bruising, breathlessness, swelling or reduced urine urgently. Flu-like symptoms cannot safely be assumed to be a harmless drug effect.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding the evidence

The national regimen brings together research and specialist guidance for a specific treatment setting.

Research results apply to the treatment schedule and patient group studied. Your team explains how the evidence applies to your own circumstances.

CHAPTER 02

Your schedule, tests and appointments

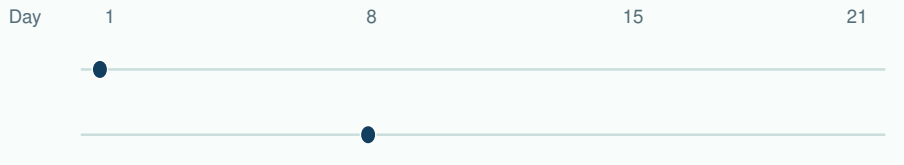
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment stages

Gemcitabine alone: nasopharyngeal cancer

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAYS 1 AND 8; EVERY 21 DAYS; UP TO 6 CYCLES.

Chemotherapy course

Up to 6 cycles

Gemcitabine 1,250 mg/m² on Days 1 and 8 every 21 days, for up to 6 cycles or until progression or unacceptable side effects.

The team reviews symptoms, side effects and response. Treatment may finish earlier if it is no longer helping or causes unacceptable problems.

BASELINE → TREATMENT → FOLLOW-UP

My tests and treatment checks

Routine safety tests, symptom-driven tests and response scans have different purposes.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00514 v3: retrieved official regimen

Tests in your plan

Full blood count

WHEN

Before each treatment.

Checks infection-fighting cells, oxygen-carrying cells and platelets for clotting.

Kidney and liver tests

WHEN

Full blood count before each treatment; kidney and liver blood tests before each cycle.

Checks treatment handling and organ safety.

THE WHOLE TREATMENT JOURNEY

Gemcitabine alone: nasopharyngeal cancer: treatment and checks

Recurrent or metastatic nasopharyngeal cancer that cannot be treated with local treatment aimed at cure.

Gemcitabine 1,250 mg/m² on Days 1 and 8 every 21 days, for up to 6 cycles or until progression or unacceptable side effects.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Next cycle / phase	After treatment
Blood and organ checks NCCP	Full blood count, kidney and liver tests; review medicines, allergies, weight, oral intake and symptoms.	Full blood count before each treatment; kidney and liver blood tests before each cycle.	Extra tests may be needed for symptoms or abnormal results.	Review results, recovery and earlier toxicity before another dose.	The team follows recovery, response and persistent side effects. Scans and tumour tests follow the individual plan; there is no universal scan or tumour-marker schedule in this regimen.
Treatment and appointments NCCP	Confirm the complete treatment plan.	Gemcitabine is given into a vein over 30 minutes on Days 1 and 8. Checks, preparation and observation add to the time in the unit.	Follow written medicine and contact instructions.	The team reviews symptoms, side effects and response. Treatment may finish earlier if it is no longer helping or causes unacceptable problems.	Record completion and the named follow-up team.
Mouth, weight and swallowing NCCP	Assess symptoms, teeth, eating and drinking.	Review weight, intake and mouth symptoms.	Call for difficulty swallowing, worsening pain or weight loss.	Dietitian and swallowing support according to need.	Continue rehabilitation and dental care if advised.

What each check is for

1 Treatment course

Days 1 and 8; every 21 days; up to 6 cycles.

Gemcitabine is used to control the cancer and relieve symptoms. Your team discusses expected benefit, alternatives and when to review whether treatment is helping.

Ask / act: Confirm active medicines, safety checks and support plan.

2

Review / next phase

After the planned course and recovery review

Review response and recovery.

Ask / act: Confirm actual dates and the next prescription.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before your first visit

Full blood count, kidney and liver tests; review medicines, allergies, weight, oral intake and symptoms.

STEP 2

Time in the unit

Gemcitabine is given into a vein over 30 minutes on Days 1 and 8. Checks, preparation and observation add to the time in the unit.

STEP 4

Leaving the unit

Confirm the next tests, medicines, treatment dates and verified daytime and out-of-hours contact numbers.

STEP 5

Practical help

Ask about transport, work, costs, communication needs and whether a support person can attend.

Your exact treatment schedule

Tests and preparation

During treatment

Full blood count before each treatment; kidney and liver blood tests before each cycle.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Gemcitabine	 **Day 1** Gemcitabine 1250 mg/m² on Day 1.
Gemcitabine	 **Day 8** Gemcitabine 1250 mg/m² on Day 8 after safety review.
Recovery and review	 **Days 9–21** Follow the support plan and report symptoms promptly.

Prescribed timing: Gemcitabine 1,250 mg/m² on Days 1 and 8 every 21 days, for up to 6 cycles or until progression or unacceptable side effects.

Safety throughout: Symptoms and urgent-contact advice apply throughout treatment and after the last dose.

Review and next step: The team reviews symptoms, side effects and response. Treatment may finish earlier if it is no longer helping or causes unacceptable problems.

> **Why these bands stay broad:** This is a reference cycle, not a personal appointment calendar. Side effects can occur on any day; the recovery interval is not a guarantee of safety. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00514 v3: retrieved official regimen ↗

> **There are no guaranteed "safe days".** Use your treating service's written temperature threshold and urgent-contact plan. Fever, shivering or sudden illness can be serious even without a high temperature. Do not wait for the next visit. Severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency help: call 112 or 999.

Between-treatment notes

Day 1: Day 1: treatment

Gemcitabine is given into a vein over 30 minutes on Days 1 and 8. Checks, preparation and observation add to the time in the unit. Report new symptoms before dosing.

Day 2–7: Between the first two visits

Follow prescribed support medicines. Blood counts and side effects may change between visits; this is not a guaranteed safe period. Call the team promptly if unwell. Keep the Day-8 blood-test and treatment appointment.

Day 8: Day 8: gemcitabine

Full blood count before each treatment; kidney and liver blood tests before each cycle. Day-8 decisions use their own blood-count and toxicity rules.

Day 9–21: Recovery and next review

Keep the next test and treatment dates. Fatigue and low blood counts may occur between visits. Do not wait for the next visit if unwell.

Support around the medicines

1 **Preventing and treating sickness**

Use the prescribed medicines for the active treatment day. The regimen is classified as low sickness risk.

2 **Mouth care and nutrition**

Follow your unit's oral-care plan. Tell the team early about mouth pain, weight loss, choking or difficulty eating and drinking. Dietetic and swallowing support can help.

3 **Medicine review**

Bring prescription, non-prescription and herbal medicines. Check interactions, allergies and any changes with the oncology pharmacist.

4 **Recovery and practical support**

Ask about fatigue, work, transport, caring responsibilities, emotional support and help with appointments.

Understanding your diagnosis

Tumour tests and safety tests

Your team explains cancer-specific tests, including Epstein–Barr virus tests where relevant in nasopharyngeal cancer. These are distinct from blood tests used for medicine safety. No universal tumour-marker schedule is prescribed here.

Your treatment aim and choices

Deciding together

Discuss expected benefit, possible lasting effects, suitable alternatives and the option of declining treatment. Tell the team what matters most to you.

Symptoms and when to call

Between visits

Keep a record of symptoms and medicines taken. Do not wait for a routine appointment when symptoms are worsening.

Mouth, eating and swallowing

Speak up early

Tell the team about pain, dry mouth, taste changes, choking, reduced intake or weight loss. Ask for a personalised oral-care, dietetic and swallowing plan.

Radiation-related care

If you are having head and neck radiation, follow the radiation team's dental, skin, saliva and swallowing advice during and after treatment. Radiation appointments and late-effect checks have their own plan.

Living with treatment

Activity and rest

Pace activity according to symptoms. Ask for help with fatigue, sleep, work, travel or caring responsibilities.

Sexual health and fertility

Discuss fertility preservation before treatment where appropriate. Ask about contraception, pregnancy, breastfeeding and medicine-specific timeframes.

Infusions and access

Your infusion arrangement

Line problems

Report pain, redness, swelling, leaking or a wet dressing. Do not adjust infusion equipment yourself; contact the unit.

After this course

Recovery and follow-up

The team follows recovery, response and persistent side effects. Scans and tumour tests follow the individual plan; there is no universal scan or tumour-marker schedule in this regimen.

Questions for your team

My plan

What is the treatment aim? Which tests are mandatory? What happens if a treatment day is missed?

My support

Which medicines do I take and when do I stop them? Whom do I contact overnight?

Questions worth asking

undefined

undefined

undefined

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undefined

Try explaining the plan back: Can you explain your treatment days, required blood tests, support medicines and whom to call if unwell? Ask the team to write down any unclear part.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Chemotherapy visit

Active medicines: Gemcitabine

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.

Medicine	Dose / route	When
Metoclopramide	10 mg PO TDS	PRN Clinician/pharmacy draft; reconcile to the individual prescription.

Additional phase-specific support

- Gemcitabine is given into a vein over 30 minutes on Days 1 and 8. Checks, preparation and observation add to the time in the unit.
- Full blood count before each treatment; kidney and liver blood tests before each cycle.
- The team reviews symptoms, side effects and response. Treatment may finish earlier if it is no longer helping or causes unacceptable problems.
- The local CUH source is retained as a reference list, not as the national antiemetic standard. Nystatin/chlorhexidine are displayed as local mouth-care adjuncts, not universal antiemetic or mucositis prophylaxis. Verify current local oral-care policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Fatigue and concentration

Gemcitabine

Tiredness can affect activity and concentration. Balance activity and rest; report sudden confusion.

2 Mouth and swallowing

Gemcitabine

Report soreness, ulcers or difficulty swallowing. Ask early about mouth care, pain relief and nutrition.

3 Breathing

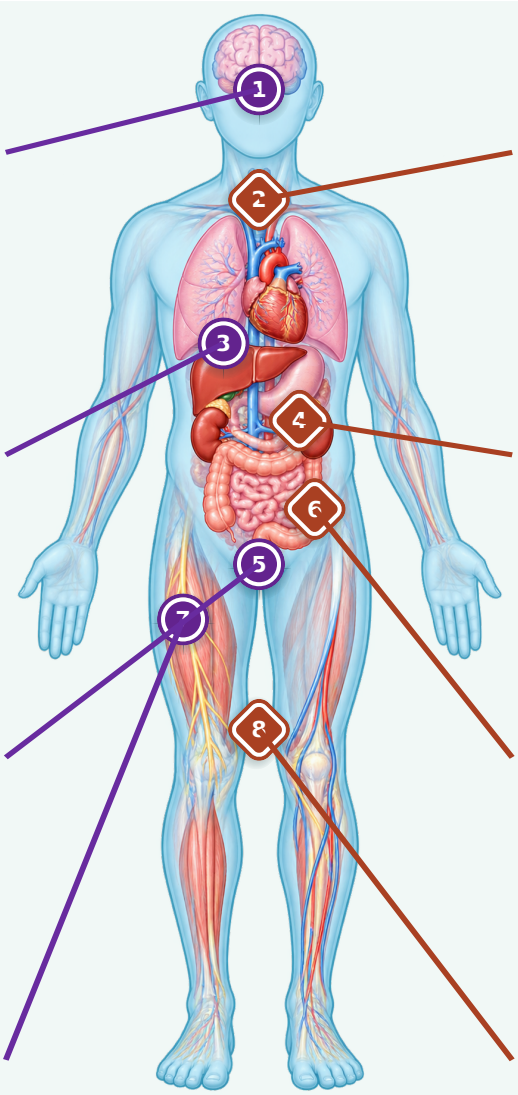
Gemcitabine

New or worsening breathlessness, cough or chest symptoms need urgent advice; lung inflammation can be serious.

4 Blood and infection

Gemcitabine

Fever, shivering, sudden illness, unexplained bruising or bleeding needs urgent oncology advice.



5 Sickness and bowels

Gemcitabine

Take prescribed anti-sickness medicines. Report persistent vomiting, diarrhoea, dehydration or inability to drink.

6 Kidneys and fluid balance

Gemcitabine

Report reduced urine, swelling or poor fluid intake. Blood tests help detect kidney injury.

7 Skin and infusion reactions

Gemcitabine

Report severe rash, blistering, facial swelling or breathing symptoms promptly. Tell staff about pain at the infusion site.

8 Fertility and pregnancy

Gemcitabine

Discuss fertility preservation, contraception, pregnancy and breastfeeding before treatment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivering, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink needs urgent oncology advice. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00514 v3: retrieved official regimen**
   https://healthservice.hse.ie/documents/6899/514_V3_Gemcitabine_1250_21day.pdf
2. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
3. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
4. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
5. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
6. **NCCP supportive-care antiemetics for NCIS**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf